

MUCOUS MEMBRANES

In the majority of patients, painful mucous membrane erosions are the presenting sign of pemphigus vulgaris (PV) and may be the only sign for an average of 5 months before skin lesions develop. However, the presenting symptoms may vary according to geographic area. For example, in a study from Croatia, painful oral lesions were the presenting symptom in only 32 percent of patients. Most of these patients progressed to a more generalized eruption within 5 months to 1 year; however, some had oral lesions for more than 5 years before generalization. On the other hand, in Tehran, 62 percent of patients presented with oral lesions.

The mucous membranes most often affected are those of the oral cavity, which is involved in almost all patients with PV and is often the only mucous membrane involved. Intact blisters are rare, likely due to their fragility and tendency to rupture easily. Scattered and often extensive erosions may appear on any part of the oral cavity, although they occur most frequently on the buccal mucosa. These erosions may spread to involve the pharynx and larynx, leading to hoarseness. Often, these erosions are so painful that the patient is unable to eat or drink adequately.

Mucous membranes in other areas may also be affected by painful erosions, including the nasal mucosa, conjunctiva, anus, penis, vagina, labia, and, in rare cases, the esophagus.

Skin involvement without mucous membrane involvement in PV is unusual and was reported in only 11 percent of cases in one study.

Pemphigus Foliaceus

SKIN

The characteristic clinical lesions of pemphigus foliaceus (PF) are scaly, crusted erosions, often on an erythematous base. In more localized and early disease, these lesions are typically well demarcated and distributed in a seborrheic pattern, affecting the face, scalp, and upper trunk.

Box 52-1: Differential Diagnosis of Pemphigus

PEMPHIGUS SUBTYPES

- Pemphigus vulgaris
- Pemphigus vegetans
- Pemphigus foliaceus
- Pemphigus erythematosus
- Endemic pemphigus foliaceus (e.g., fogo selvagem)
- Immunoglobulin A (IgA) pemphigus
- Subcorneal pustular dermatosis
- Intraepidermal neutrophilic dermatosis
- Paraneoplastic pemphigus

INTRAEPIDERMAL BLISTERING DISEASES WITHOUT AUTOANTIBODIES

- Familial benign pemphigus (Hailey-Hailey disease)
- Bullous impetigo, staphylococcal scalded-skin syndrome

- Blisters from herpes simplex and zoster
- Allergic contact dermatitis (e.g., rhus dermatitis)
- Epidermolysis bullosa simplex
- Incontinentia pigmenti

MOUTH ULCERS/EROSIONS WITHOUT AUTOANTIBODIES

- Aphthous ulcers
- Candidiasis
- Lichen planus
- Behçet disease

SUBEPIDERMAL BLISTERING DISEASES WITH AUTOANTIBODIES

- Bullous pemphigoid
- Herpes gestationis
- Cicatricial pemphigoid
- Epidermolysis bullosa acquisita
- Linear IgA disease and chronic bullous disease of childhood
- Dermatitis herpetiformis
- Bullous lupus erythematosus

SUBEPIDERMAL BLISTERING DISEASES WITHOUT AUTOANTIBODIES

- Erythema multiforme
- Toxic epidermal necrolysis
- Porphyria
- Epidermolysis bullosa